

Safety

TC: C-1117 (TC)

[Link to Codes](#)

MCG Health

Transitions of Care
28th Edition

- Assessment
- Plan of Care
- Evidence Summary
- References
- Footnotes
- Codes

Assessment

Q1. Patient: Are there any safety risks that may lead to problems being safe at home? (Check all that apply.)

- Accessibility (eg, outside or inside stairs) [Q2] [P1]
- Dizziness [Q2] [P1]
- Falls [Q2] [P1]
- Inside home safety hazards (eg, halls and rooms are cluttered, inadequate lighting, bathroom unsafe, stairs without rails, pets) [Q2] [P1]
- Memory or cognition [Q2] [P1]
- Outside home safety hazards (eg, doors that do not lock, stairs without rails) [Q2] [P1]
- Polypharmacy (4 or more medicines) [Q2] [P1]
- Problems with mobility or transfers [Q2] [P1] [Activities of Daily Living](#) [TC](#)
- Substances used that increase risk (eg, benzodiazepines, sedative-hypnotics, antidepressants, antipsychotics, alcohol, opiates)
- Vision problems [Q2] [P1]
- Other safety concerns: _____ [Q2] [P1]
- None
- Unknown [Q2]

Q2. Patient: Is there a plan in place to manage home safety risks? If yes, what is the plan?

- Yes, specify: _____ [P1]
- No [P2]
- Unknown [P1]
- Not applicable

Q3. Patient: Are you concerned about your safety?

- Yes, specify: _____ [P1] [P2] [Intimate Partner Violence](#) [TC](#)
- No
- Unknown [P1]

Q4. Patient: Has a nurse or physical therapist ever come to your home to do a home safety assessment?

- Yes, specify date: _____
- No [P1]
- Unknown [P1]

Plan of Care

Problems

P1. Home safety management information needed

- **Goal:** Home safety management plan understood and followed(1)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)

- **ASSIST:** Appointment scheduling for evaluation of dizziness, unexplained falls, memory loss, medication issues, or other home safety concerns^[N]
- **ASSIST:** Identifying patient's needs, abilities, preferences, and risks for problems. See Home Safety Assessment [SR](#) for further information.^{[A](2)}
- **ASSIST:** Medication list and pharmacy review⁽³⁾
- **ASSIST:** Identifying environmental structural problems or modifications (eg, steps, lighting, access to bathroom or kitchen)⁽⁴⁾
- **ASSIST:** Obtaining emergency medical identification, if needed
- **ASSIST:** Acquiring and using devices for communication (eg, cell phone, alarm system) in case of emergency⁽⁵⁾
- **ASSIST:** Identifying and moving to different living arrangements, as needed
- **ASSIST:** Creating emergency evacuation plan, as needed
- **EDUCATE:** Home changes to help prevent falls:
 - Remove all throw rugs and clutter from floors.
 - Repair carpet tears, broken tiles, or hardwood floor ruts that could cause tripping.
 - Clear walking areas of electrical cords or wires.
 - Move furniture to allow for use of crutches or walker.
 - Keep all areas well lit.⁽⁶⁾
 - Clean up spills immediately.
- **EDUCATE:** Self-care safety activities⁽⁷⁾⁽⁸⁾:
 - Rise slowly from lying or sitting position.
 - Wear glasses or contact lenses, if needed.
 - Use handrails, grab bars, or adaptive devices as needed.
 - Wear shoes with nonslip soles.
 - Use no-slip mats in shower or tub.
 - Pet-related falls mitigation (eg, not chasing pet, obedience training)
 - Know which medication may cause problems (eg, dizziness, weakness, or balance problems).
 - Exercise program within provider's recommendations⁽⁴⁾⁽⁹⁾^[I]
- **EDUCATE:** Safety hazard removal and ways to cut down safety risks⁽⁸⁾:
 - Architectural changes that can improve safety (eg, widening doors to allow for wheelchair, putting grab bars in bathroom)
 - Hot water tank temperature setting between 110 to 120 degrees F (43.4 to 48.9 degrees C)⁽¹⁰⁾
 - Snow and ice removal plan
 - Chairs and other seats stable and at height to facilitate easy rising
 - Electrical appliances, cords, and outlets safe, used appropriately, and in good repair
- **EDUCATE:** Fire prevention:
 - Smoke detector functioning, and plan to change battery regularly
 - Fireplace in good condition, with screen and chimney cleaned regularly
 - Smoking habits as safe as possible
 - Electrical and heating units away from flammable material
 - Electrical appliances, cords, and outlets safe, used appropriately, and in good repair
 - Evacuation plan in event of fire that includes location of available exits
 - How to put out flames if clothes catch on fire
- **EDUCATE:** When to get emergency help:
 - Fall complications (eg, head injury, broken bone, significant ADL changes)
 - Fear of personal harm from someone (eg, spouse, parent). See Intimate Partner Violence [TC](#).
 - Stroke, heart attack, or blood clot
- **EDUCATE:** When to call the provider:
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Depression (eg, feeling sad, hopeless, overwhelmed)⁽¹¹⁾
 - Falls
 - Medication problems (eg, side effects, access, administration)
 - Self-care changes (eg, ADL or IADL problems)
 - Physical activity level declines (eg, fatigue, weakness).
- **COORDINATE:** Pharmacy medication support, if indicated^{[B](12)}
- **COORDINATE:** Specialist referral, if indicated
- **COORDINATE:** Home care referral, if indicated⁽¹³⁾
- **COORDINATE:** Rehabilitation therapy referral, if indicated⁽⁴⁾⁽¹³⁾
- **COORDINATE:** Medical equipment and supplies, if indicated⁽¹⁴⁾
- **COORDINATE:** Community services referral, as needed^[C]
- **COORDINATE:** Social worker referral, as needed⁽¹⁵⁾
- **COORDINATE:** Protective services or legal authorities, as needed⁽¹²⁾
- **COORDINATE:** Emergency plans for regional weather or other emergencies (eg, earthquakes, floods, tornadoes)

- **COORDINATE:** Follow-up contact to evaluate safety, referral status, and need for further assistance(16)(17)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Exercising to improve balance (Illustrated)  (Spanish)
- **SEND:** Exercising when you have neurologic problems  (Spanish)
- **SEND:** How to get out of bed safely (Illustrated)  (Spanish)
- **SEND:** How to move safely in bed (Illustrated)  (Spanish)
- **SEND:** How to use a cane (Illustrated)  (Spanish)
- **SEND:** How to use a walker (Illustrated)  (Spanish)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)
- **SEND:** Safe lifting (Illustrated)  (Spanish)
- **SEND:** Safety in the home - Fall protection  (Spanish)
- **SEND:** Safety in the home - Fire and other risks  (Spanish)
- **SEND:** Safety in the home - Memory loss  (Spanish)
- **SEND:** Walking with crutches (Illustrated)  (Spanish)
- **SEND:** Ways to save energy  (Spanish)

P2. Home safety issues not managed

- **Goal:** Safety issues identified and addressed(18)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of dizziness, unexplained falls, memory loss, medication issues, or other home safety concerns
 - **ASSIST:** Contact with legal authorities, as appropriate
 - **EDUCATE:** When and how to get help for worsening problems or concerns for safety
 - **COORDINATE:** Social worker referral, as needed(15)
 - **COORDINATE:** Medical equipment and supplies, if indicated(14)
 - **COORDINATE:** Protective services or legal authorities, as needed
 - **COORDINATE:** Pharmacy medication support, if indicated(12)
 - **COORDINATE:** Rehabilitation therapy referral, if indicated(4)(13)
 - **COORDINATE:** Specialist referral for comorbidities, if indicated
 - **COORDINATE:** Audiologist referral, if indicated
 - **COORDINATE:** Ophthalmologist referral, if indicated
 - **COORDINATE:** Home care referral, if indicated(13)
 - **COORDINATE:** Follow-up contact to evaluate safety issues, referral status, and need for further assistance(16)(17)

Evidence Summary

Evidence: Multiple studies indicate that the large number and types of medications consumed by the elderly increase the risk of falls and should be evaluated.(3)(18)(19)

National guidelines: The U.S. Preventive Services Task Force recommends exercise-based interventions to help reduce falls for people 65 years and older. Other interventions (eg, behavioral therapy, nutrition therapy, environmental modification, medication changes) may be considered based on the patient's preferences, the benefit-to-harm balance, and the circumstances of prior falls. Vitamin D supplementation is not recommended for fall prevention for older people without a known deficiency or osteoporosis.(4)

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Footnotes

[A] Fall risk factors include mobility or functional impairment, poor health, weakness, incontinence, polypharmacy (4 or more medications), medication dose changes, or patient taking substances that increase risk (eg, benzodiazepines, sedative-hypnotics, antidepressants, antipsychotics, alcohol, opiates).(18)(20) [A in Context Link 1]

[B] A pharmacist can support medication reconciliation for redundancy or duplication and identify drug interactions (eg, with other drugs, food).(12) [B in Context Link 1]

[C] Services to consider for community support include stairway modifications, widening of interior doors for ambulation equipment, and installation of handrails.(1) [C in Context Link 1]

Codes

ICD-10 Diagnosis: Z59.01, Z59.3, Z59.811, Z59.812, Z59.819, Z60.2

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